

Session Objectives

1. Students will be able to diagnose the innominates and locate anterior pelvic counterstrain points
2. Students will be able to perform HVLA for an anteriorly and posteriorly rotated innominate, and superior and inferior innominate shears, all preceded by the use of soft tissue techniques
3. Students will be able to discuss the principles of counterstrain, including discussing the anatomical correlations and clinical indications of the pelvic counterstrain points.
4. Students will correctly perform counterstrain for the treatment of the following anterior pelvic counterstrain points: iliacus, psoas, low ilium and inguinal

Counterstrain (CS):

- Counterstrain points are tenderpoints that can be treated with counterstrain.
- Passive Indirect technique, making it extremely gentle and versatile.
- Originally called, “spontaneous release by positioning.”

Clinical pointers (worthwhile to remember these in practice):

- Due to its gentle nature, CS is very safe and versatile when properly performed for the right patient.
- Can be used in addition to other techniques to enhance results if needed.
- Patient’s symptoms may not be in the region of the CS point(s) but yet may be caused by them, so it is important to recognize when you should check for them.

Counterstrain treatment: Basic steps (reminders and tips)

1. Find the tender point with your monitoring finger. **Remember that the keys are that the point is tender, and to use your monitoring finger to detect soft tissue changes to guide you to the point (in practice, the location descriptions of the points are useful but are not the final determination of where the point is exactly located!)**
2. Set a relative pain scale for the patient by pressing on the point and calling it a 10/10. **This is not meant to rate the point more painful than it actually is. You are simply giving the patient an easy to remember starting point of 10/10.**
 - You can say something like, “We are going to call the pain you feel when I press on this point a 10 out of 10 in severity.”
3. Slowly place the patient passively in a position of ease that results in the greatest reduction of tenderness at the CS point (**“Fold and Hold” concept**).
 - a) You maintain the position of your finger on the point but ease up on your finger pressure while monitoring as you perform the counterstrain positioning to achieve this ideal position.
 - b) You press on the point to test the effectiveness of the treatment position and ask the patient “How does it feel now based the 10 we started with?” Remember to relax your finger pressure once the patient reports the tenderness.
 - c) FINE TUNE the position to achieve the greatest position of ease. **Aim to achieve a 0/10 rating (i.e. 100% reduction of tenderness). If it is not 0/10 then you should fine-tune further until at least a 3/10 rating is achieved. This may take one or more tries, but the idea is that you should make a reasonable number of attempts to achieve the maximal reduction in tenderness whenever possible. For example, if you fine-tune and get a 3/10 on the first try, why not try it again to see if you can reduce it further to a 2/10 or lower- make the 90 seconds you will spend in the treatment position count!**

Counterstrain treatment: Basic steps (reminders and tips)

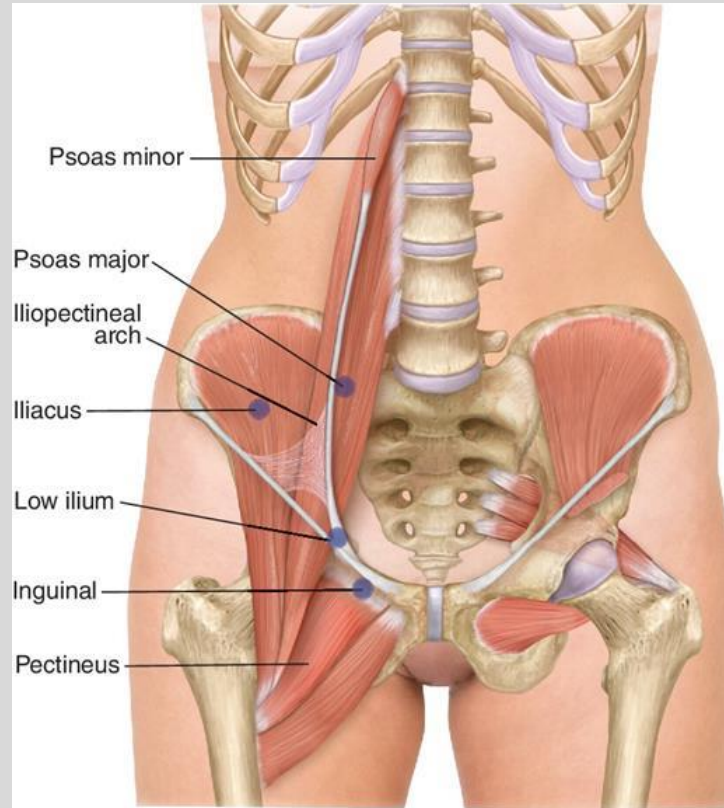
7. Hold the final treatment position for 90 seconds while monitoring the point.
 8. At the end of 90 seconds, slowly return the patient to the pre-treatment position *while maintaining contact* with the counterstrain point.
 - Prior to moving the patient, instruct the patient to “relax and do not help me”, then slowly return the patient to the initial neutral position. These steps will help avoid exciting the muscle spindles.
 9. REASSESS the CS point for tenderness and tissue texture abnormality. **Press on the point and ask the patient, again, “How does it feel now based the 10 we started with?” You are looking for a 0/10. Achieving a 3/10 or less is considered a successful treatment of that point. If the tenderness is >3/10, then you may need or want to re-treat it with CS.**
- ❖ Note that when we begin the technique, we **assign** the tenderness as 10/10 in order to have an easy to remember starting point. During the technique, as well as when reassessing after completing the technique, we are asking the patient to rate the tenderness on a 0-10 scale relative to the initially assigned level of 10. Again, consider retreating if tenderness is greater than 3 on the recheck.

Additional pointers for this lab:



1. Legs are heavy!- Use proper set up such as appropriate table height, correct posture, and good control because these are all important to perform the hip and pelvic CS techniques comfortably and efficiently.
2. Gauge if your partner is truly relaxed. If the legs feel unusually light as you are holding them, it probably means that your partner is not completely relaxed and is holding the leg up for you.
3. Use the CS point scale- that is how you determine whether you have achieved the mobile point (i.e. ≤ 3 tenderness); remember to make a few attempts to get to 0/10 even though this is not always possible. Make sure your patient remains comfortable during this process (i.e. avoid causing pain in CS point region or other region as you try to get to a 0/10; it's often not an issue but sometimes you have to compromise on how much you want to reduce the tenderness to). This is why you use small movements when fine-tuning.
4. Hip and knee arthritis are common in older patients, so be mindful how far you move the knees and hips when performing CS. Always tailor your treatment to your patient's unique needs.
5. Fine tuning is exactly what it sounds like- small incremental movements. Remember that combining different motions (e.g. flexion with external rotation) may allow you to achieve the mobile point more quickly and comfortably, without overusing the motion available in any one plane (which can cause discomfort if you move it too far).
6. As you perform CS, tune into your own body mechanics and get a sense of how it feels. You will realize that the described treatment positions are designed to allow you maximal control with the least amount of effort (if you doubt this, you can *as a learning exercise* change your hand or body position slightly and sense how it feels to you— you may discover that it doesn't feel as natural or may even be uncomfortable, in which case you should immediately stop and readjust your body and hand position).

Pelvic Counterstrain points: **Anterior**



****You are only responsible for the anterior pelvic CS points in this lab that are noted on the session objectives (see CPG or this PPT)**

****Please also learn the anatomical correlation(s) of each CS point**

Before Performing HVLA

- Before HVLA, you must first make a correct diagnosis and perform a soft-tissue technique such as Muscle Energy, FPR, CS, Soft-tissue stretching etc. to release the muscles and/or fascia around the area being treated.
 - **Review the prior lab for how to perform Pelvic Diagnosis.***
 - **It is also recommended to review MET for Anterior, posterior, superior and inferior innominate somatic dysfunctions before proceeding. This will reinforce your understanding of pelvic mechanics, AND allow you to choose MET (if that is what you want to use) as the preparatory technique prior to innominate HVLA.***

Reminders

Review innominate diagnosis- aim to be able to make an accurate diagnosis in an efficient manner. Hint: remember to name landmark findings based on side of +standing flexion test; reseat the pelvis to increase accuracy